Substance Use Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

A substance use disorder (SUD) is a problematic pattern of using alcohol or another drug that continues despite clear harm. DSM-5-TR uses one set of 11 criteria across substances (alcohol, cannabis, opioids, stimulants, sedatives, and others) and rates severity by how many are met (American Psychiatric Association [APA], 2022).

How common: in the U.S., lifetime alcohol use disorder was 29.1% (13.9% in the past year), yet **only 19.8% had ever been treated** (Grant et al., 2015). Lifetime drug use disorder was 9.9% (3.9% in the past year) (Grant et al., 2016).

Rarely alone: across 22 surveys, people with SUDs had much higher odds of depression and anxiety; illicit drug use disorder carried nearly four times the odds of major depression (OR 3.80) (Lai et al., 2015).

Counselor framing: addiction is a treatable condition, not simply a moral failure. Most people with an SUD never receive treatment, so every counselor and pastor needs to screen for it.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A problematic pattern causing distress or impairment, with **2 or more of 11** within 12 months:

- **Impaired control:** using more or longer than intended; wanting or failing to cut down; much time spent getting, using, or recovering; craving.
- **Social impairment:** failing major duties at work, school, or home; continuing despite relationship problems; giving up important activities.
- **Risky use:** use in physically dangerous situations; continuing despite known physical or psychological harm.
- **Physical dependence:** tolerance and withdrawal (these two do not count when a medication is taken as prescribed).
- **Severity:** mild (2 to 3 criteria), moderate (4 to 5), severe (6 or more). Specifiers include early or sustained remission and "in a controlled environment."

Screening: the 3-question AUDIT-C detected heavy drinking as well as the full 10-item AUDIT (AUROC 0.891 vs. 0.881) (Bush et al., 1998). Ask about every substance, including prescription medications.

Suicide risk: suicide rates were about 10 times expected in alcohol use disorder, 13.5 times in opioid use disorder, and nearly 17 times with mixed drug use (Wilcox et al., 2004). Ask directly, especially during intoxication, withdrawal, and relapse.

When counselors must refer (general clinical consensus): refer for medical care when there is risk of dangerous withdrawal (alcohol, benzodiazepines), opioid use (overdose risk; medication treatment), pregnancy, suicidality, or serious medical or psychiatric illness.

3. What works: treatment and the evidence

Bottom line for class: several approaches work about equally well: CBT and relapse prevention, motivational interviewing, contingency management, and 12-step facilitation. For opioid and alcohol use disorders, medication adds real benefit, and for opioids it saves lives.

CBT (honest effect sizes):

- Across 53 trials, CBT had a small overall effect (g = 0.15) that faded by 12 months, but a large effect against no treatment (g = 0.80) and for cannabis (g = 0.51) (Magill & Ray, 2009).
- An updated meta-analysis found CBT beat minimal or no treatment by a moderate margin, but was **not better than other specific therapies** such as motivational or 12-step approaches (Magill et al., 2019).
- Relapse prevention (identifying high-risk situations and coping with them) was effective, especially for alcohol and when combined with medication (Irvin et al., 1999).

Other approaches. Psychosocial treatments for drug use have moderate effects overall, and **contingency management** (rewards for negative drug tests) had the strongest (Dutra et al., 2008): d = 0.42 overall and about 0.65 for opioids and cocaine (Prendergast et al., 2006). **Motivational interviewing** produced small, durable effects against weak comparisons (g = 0.28) but was not better than other specific treatments (Lundahl et al., 2010).

12-step programs. In a Cochrane review of 27 studies (10,565 people), manualized AA and twelve-step facilitation **improved continuous abstinence at 12 months** compared with other treatments such as CBT (RR 1.21, high-certainty evidence) (Kelly et al., 2020).

Medication (referral notes). For alcohol, naltrexone (number needed to treat 12 to prevent heavy drinking) and acamprosate (12 to prevent any drinking) help (Jonas et al., 2014). In the COMBINE trial, naltrexone or a combined behavioral intervention each improved abstinence alongside medical management (Anton et al., 2006). For opioids, buprenorphine keeps people in treatment and at high doses reduces illicit use (Mattick et al., 2014); death rates were about 3 times higher out of methadone treatment than in it (Sordo et al., 2017). **Never discourage medication for opioid use disorder.**

CBT for SUDs: how it is used:

- **Build motivation** before change; resolve ambivalence (Lundahl et al., 2010).
- **Map triggers and high-risk situations**, and practice coping and refusal skills (Irvin et al., 1999).
- **Challenge permission-giving and hopeless beliefs**: "I can handle just one," "I deserve it," "I can't cope without it," "I've already blown it, so I might as well keep going" (Magill et al., 2019).
- **Plan for lapses** so a slip does not become a full relapse, and connect the person to ongoing peer support (Kelly et al., 2020).

4. Religion, spirituality, and Christian integration: what the evidence shows

Faith is already part of most treatment. About 73% of U.S. addiction treatment programs include a spiritual element, mostly through 12-step programs that emphasize reliance on God or a Higher Power (Grim & Grim, 2019).

Spirituality helps explain how recovery works. In AA, attending meetings increased spiritual practices, and this growth partly explained AA's better drinking outcomes (Kelly et al., 2011).

Intervention studies:

- **Spiritual and religious interventions overall** (20 randomized trials, 3,700 people): a small but significant advantage over other treatments (d = 0.18). Because only 12-step approaches were compared head to head, this does not yet show that other faith-based treatments are superior (Hai et al., 2019).
- **Christian faith-based treatment** (not randomized): compared with a secular program, residents of a Christian program grew more in religiosity, and that growth helped explain their abstinence 6 months after treatment (Yeung, 2022).
- **CBT in the church:** a computer-delivered CBT program led by church health advisors in Black churches (n = 40) was feasible, with good retention and reduced substance use; a randomized trial is still needed (Jordan et al., 2021).

Honest summary for teaching:

1. CBT, motivational interviewing, contingency management, 12-step facilitation, and (for alcohol and opioids) medication are the evidence base. Faith integration supports them; it does not replace them.
2. Spiritual growth is one of the ways recovery works, and 12-step approaches have the strongest evidence among faith-oriented treatments.
3. Christian-specific programs show promising but non-randomized results. Combine them with evidence-based care and medical treatment, never in place of it.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within CBT and relapse prevention. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Ask about faith as a resource and as a source of shame. Listen for beliefs such as "God could never forgive what I've done," "If I had enough faith I wouldn't need treatment or medication," and "I've relapsed, so I'm not really a Christian."
- **Challenge (clinical).** Test permission-giving thoughts and shame with evidence and a lapse plan. Correct the belief that medication for opioid use disorder is "trading one drug for another" (Mattick et al., 2014; Sordo et al., 2017).

- **Replace (pastoral).** Offer Scripture of freedom, forgiveness, and strength in temptation:
  - "No temptation has seized you except what is common to man. And God is faithful; He will not let you be tempted beyond what you can bear. But when you are tempted, He will also provide an escape, so that you can stand up under it." (1 Corinthians 10:13). Build the "escape" into the relapse plan.
  - "Therefore if anyone is in Christ, he is a new creation. The old has passed away. Behold, the new has come!" (2 Corinthians 5:17)
  - "Because of the loving devotion of the LORD we are not consumed, for His mercies never fail. They are new every morning; great is Your faithfulness!" (Lamentations 3:22–23). A lapse is not the end.
  - "Therefore confess your sins to each other and pray for each other so that you may be healed." (James 5:16). Accountability and peer support.
- **Faith community (clinical).** Connect the person with a recovery group, such as AA or a church-based recovery ministry, and a sponsor or accountability partner (Kelly et al., 2020; Kelly et al., 2011).
- **Ministers:** screen with the AUDIT-C, respond without shame, and refer for medical assessment before anyone stops alcohol, sedatives, or opioids suddenly.

6. Quick classroom takeaways

- Diagnose by 2 or more of 11 criteria in a year; rate mild, moderate, or severe. Screen everyone (AUDIT-C), and ask about suicide and overdose risk.
- CBT, motivational interviewing, contingency management, and 12-step facilitation all work; medication is essential for opioid use disorder and helpful for alcohol use disorder.
- Faith and fellowship are part of how recovery works. Replace shame and "I've blown it" thinking with the truth of God's forgiveness and new mercies.

Sources note

All 22 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Anton, R. F., O'Malley, S. S., Ciraulo, D. A., Cisler, R. A., Couper, D., Donovan, D. M., Gastfriend, D. R., Hosking, J. D., Johnson, B. A., LoCastro, J. S., Longabaugh, R., Mason, B. J., Mattson, M. E., Miller, W. R., Pettinati, H. M., Randall, C. L., Swift, R., Weiss, R. D., Williams, L. D., . . . COMBINE Study Research Group. (2006). Combined pharmacotherapies and behavioral interventions for alcohol dependence: The COMBINE study: A randomized controlled trial. *JAMA*, *295*(17), 2003–2017. https://doi.org/10.1001/jama.295.17.2003

Bush, K., Kivlahan, D. R., McDonell, M. B., Fihn, S. D., & Bradley, K. A. (1998). The AUDIT alcohol consumption questions (AUDIT-C): An effective brief screening test for problem drinking. *Archives of Internal Medicine*, *158*(16), 1789–1795. https://doi.org/10.1001/archinte.158.16.1789

Dutra, L., Stathopoulou, G., Basden, S. L., Leyro, T. M., Powers, M. B., & Otto, M. W. (2008). A meta-analytic review of psychosocial interventions for substance use disorders. *American Journal of Psychiatry*, *165*(2), 179–187. https://doi.org/10.1176/appi.ajp.2007.06111851

Grant, B. F., Goldstein, R. B., Saha, T. D., Chou, S. P., Jung, J., Zhang, H., Pickering, R. P., Ruan, W. J., Smith, S. M., Huang, B., & Hasin, D. S. (2015). Epidemiology of DSM-5 alcohol use disorder: Results from the National Epidemiologic Survey on Alcohol and Related Conditions III. *JAMA Psychiatry*, *72*(8), 757–766. https://doi.org/10.1001/jamapsychiatry.2015.0584

Grant, B. F., Saha, T. D., Ruan, W. J., Goldstein, R. B., Chou, S. P., Jung, J., Zhang, H., Smith, S. M., Pickering, R. P., Huang, B., & Hasin, D. S. (2016). Epidemiology of DSM-5 drug use disorder: Results from the National Epidemiologic Survey on Alcohol and Related Conditions-III. *JAMA Psychiatry*, *73*(1), 39–47. https://doi.org/10.1001/jamapsychiatry.2015.2132

Grim, B. J., & Grim, M. E. (2019). Belief, behavior, and belonging: How faith is indispensable in preventing and recovering from substance abuse. *Journal of Religion and Health*, *58*(5), 1713–1750. https://doi.org/10.1007/s10943-019-00876-w

Hai, A. H., Franklin, C., Park, S., DiNitto, D. M., & Aurelio, N. (2019). The efficacy of spiritual/religious interventions for substance use problems: A systematic review and meta-analysis of randomized controlled trials. *Drug and Alcohol Dependence*, *202*, 134–148. https://doi.org/10.1016/j.drugalcdep.2019.04.045

Irvin, J. E., Bowers, C. A., Dunn, M. E., & Wang, M. C. (1999). Efficacy of relapse prevention: A meta-analytic review. *Journal of Consulting and Clinical Psychology*, *67*(4), 563–570. https://doi.org/10.1037/0022-006X.67.4.563

Jonas, D. E., Amick, H. R., Feltner, C., Bobashev, G., Thomas, K., Wines, R., Kim, M. M., Shanahan, E., Gass, C. E., Rowe, C. J., & Garbutt, J. C. (2014). Pharmacotherapy for adults with alcohol use disorders in outpatient settings: A systematic review and meta-analysis. *JAMA*, *311*(18), 1889–1900. https://doi.org/10.1001/jama.2014.3628

Jordan, A., Babuscio, T., Nich, C., & Carroll, K. M. (2021). A feasibility study providing substance use treatment in the Black church. *Journal of Substance Abuse Treatment, 124*, 108218. https://doi.org/10.1016/j.jsat.2020.108218

Kelly, J. F., Stout, R. L., Magill, M., Tonigan, J. S., & Pagano, M. E. (2011). Spirituality in recovery: A lagged mediational analysis of Alcoholics Anonymous' principal theoretical mechanism of behavior change. *Alcoholism: Clinical and Experimental Research*, *35*(3), 454–463. https://doi.org/10.1111/j.1530-0277.2010.01362.x

Kelly, J. F., Humphreys, K., & Ferri, M. (2020). Alcoholics Anonymous and other 12-step programs for alcohol use disorder. *Cochrane Database of Systematic Reviews*, *2020*(3), CD012880. https://doi.org/10.1002/14651858.CD012880.pub2

Lai, H. M. X., Cleary, M., Sitharthan, T., & Hunt, G. E. (2015). Prevalence of comorbid substance use, anxiety and mood disorders in epidemiological surveys, 1990–2014: A systematic review and meta-analysis. *Drug and Alcohol Dependence*, *154*, 1–13. https://doi.org/10.1016/j.drugalcdep.2015.05.031

Lundahl, B. W., Kunz, C., Brownell, C., Tollefson, D., & Burke, B. L. (2010). A meta-analysis of motivational interviewing: Twenty-five years of empirical studies. *Research on Social Work Practice*, *20*(2), 137–160. https://doi.org/10.1177/1049731509347850

Magill, M., & Ray, L. A. (2009). Cognitive-behavioral treatment with adult alcohol and illicit drug users: A meta-analysis of randomized controlled trials. *Journal of Studies on Alcohol and Drugs*, *70*(4), 516–527. https://doi.org/10.15288/jsad.2009.70.516

Magill, M., Ray, L., Kiluk, B., Hoadley, A., Bernstein, M., Tonigan, J. S., & Carroll, K. (2019). A meta-analysis of cognitive-behavioral therapy for alcohol or other drug use disorders: Treatment efficacy by contrast condition. *Journal of Consulting and Clinical Psychology*, *87*(12), 1093–1105. https://doi.org/10.1037/ccp0000447

Mattick, R. P., Breen, C., Kimber, J., & Davoli, M. (2014). Buprenorphine maintenance versus placebo or methadone maintenance for opioid dependence. *Cochrane Database of Systematic Reviews*, *2014*(2), CD002207. https://doi.org/10.1002/14651858.CD002207.pub4

Prendergast, M., Podus, D., Finney, J., Greenwell, L., & Roll, J. (2006). Contingency management for treatment of substance use disorders: A meta-analysis. *Addiction, 101*(11), 1546–1560. https://doi.org/10.1111/j.1360-0443.2006.01581.x

Sordo, L., Barrio, G., Bravo, M. J., Indave, B. I., Degenhardt, L., Wiessing, L., Ferri, M., & Pastor-Barriuso, R. (2017). Mortality risk during and after opioid substitution treatment: Systematic review and meta-analysis of cohort studies. *BMJ*, *357*, j1550. https://doi.org/10.1136/bmj.j1550

Wilcox, H. C., Conner, K. R., & Caine, E. D. (2004). Association of alcohol and drug use disorders and completed suicide: An empirical review of cohort studies. *Drug and Alcohol Dependence*, *76*(Suppl.), S11–S19. https://doi.org/10.1016/j.drugalcdep.2004.08.003

Yeung, J. W. K. (2022). Faith-based intervention, change of religiosity, and abstinence of substance addicts. *Brazilian Journal of Psychiatry*, *44*(1), 46–56. https://doi.org/10.1590/1516-4446-2020-1576

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